ENGLISH FOR WORK / SEPTEMBER 2026

Pharmaceutical English

Vocabulary & phrasebook

Keep precise meanings and useful expressions close. Retrieve the words, then use them in a complete message.

PRACTICE THAT TRANSFERS TO WORK

Read a realistic case. Study complete conversations. Find the right language, speak, get feedback, and try again.

Eight lessons | Intermediate to advanced | Independent or classroom study

For clinical development, regulatory affairs, pharmacovigilance, quality, CMC, manufacturing, medical affairs, market access, and compliance teams.

Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

All scenarios, figures, and model responses are fictional teaching material. Use invented details in practice.

Open this course on English Ladder

Make vocabulary usable

Knowing a definition is a start. To use a term well, explain it in plain English, connect it to a case, and choose a natural sentence around it.

- Read five terms, cover their definitions, and recall the meanings.
- Sort a pair you might confuse and explain the difference.
- Use one term in a sentence about a fictional case.
- Ask a partner to explain the sentence without repeating the specialist word.
- Return to the same terms on another day and test recall again.

Abbreviations need context

Expand an abbreviation on first use when the listener needs it. Some abbreviations have different meanings across professions: API can refer to an application programming interface or an active pharmaceutical ingredient. The course context determines the meaning.

Definitions have limits

These are concise language-learning explanations, not complete legal, clinical, or technical specifications. Use the applicable authoritative definition for regulated or operational decisions.

Field vocabulary A-Z

Accelerated approval
Approval pathway using a surrogate or intermediate clinical endpoint reasonably likely to predict clinical benefit.

AE
Adverse event; unfavorable medical occurrence after product use, regardless of causality.

API
Active pharmaceutical ingredient: the component intended to provide a medicine's pharmacological activity.

Batch record
Documented record of manufacturing steps, controls, and results for a batch.

Biosimilar
Biologic highly similar to a reference product with no clinically meaningful differences.

BLA
Biologics license application requesting approval to market a biologic.

Blinding
Keeping treatment assignment unknown to reduce bias.

CAPA
Corrective and preventive action addressing root cause and recurrence prevention.

Case narrative
Written summary of an individual safety case.

CMC
Chemistry, manufacturing, and controls information supporting product quality.

Complete response letter
FDA communication that an application is not ready for approval in its current form.

Confidence interval
An interval produced by a method with a stated long-run coverage rate under its assumptions.

Control arm
Comparator group used to interpret the effect of the investigational treatment.

Data integrity
Completeness, consistency, accuracy, and reliability of data throughout the lifecycle.

Deviation
Departure from approved procedure, process, specification, or expected result.

Estimand
Precise description of the treatment effect being estimated.

Exploratory endpoint
Outcome assessed to generate hypotheses or additional insight, usually not definitive.

Fair balance
Balanced communication of benefits and risks so the message is not misleading.

Formulary
List of medicines covered or preferred by a payer or health system.

HEOR
Health economics and outcomes research.

IND
Investigational new drug application allowing clinical investigation in humans in the United States.

Indication
Disease, condition, or patient population for which a product is intended or approved.

Indication statement
Approved language describing what the drug is approved to treat and in whom.

Informed consent
Process and documentation showing that participants understand key trial information before participation.

Intercurrent event
Event after treatment initiation that affects interpretation or existence of measurements.

Lifecycle management
Post-approval strategy for evidence, indications, formulations, access, safety, and competition.

MedDRA
Medical Dictionary for Regulatory Activities used to code medical terms.

Medical information

Function that responds to medical inquiries with approved, balanced, evidence-based information.

MLR

Medical, legal, and regulatory review of materials.

MOA

Mechanism of action; how the product is understood to produce a biological effect.

NDA

New drug application requesting approval to market a drug.

Off-label

Use outside the approved product labeling; the applicable legal and professional context must be distinguished from promotional activity.

OOS

Out of specification result requiring investigation.

P-value

Measure of compatibility between observed data and a null hypothesis under model assumptions.

Prescribing information

Approved product information describing indication, dosing, warnings, adverse reactions, and other use information.

Primary endpoint

Main outcome measure used to evaluate the primary objective.

Prior authorization

Payer requirement for approval before coverage.

Process validation

Evidence that a manufacturing process consistently produces product meeting quality requirements.

Promotional claim

Product-related statement intended to promote use and requiring support and compliance review.

Protocol
Document describing study objectives, design, population, assessments, endpoints, safety, and analysis.

Protocol deviation
Departure from the approved protocol or study procedures.

Randomization
Assignment to treatment groups by chance to reduce bias.

REMS
Risk Evaluation and Mitigation Strategy used to manage serious known or potential risks when required.

Risk-based monitoring
Monitoring approach focused on critical risks to participant safety and data reliability.

Risk-benefit
Integrated assessment of benefits, risks, uncertainty, disease severity, and alternatives.

RWD
Real-world data routinely collected from health care or patient sources.

RWE
Clinical evidence about usage, benefits, or risks derived from RWD analysis.

SAE
Serious adverse event meeting criteria such as death, life-threatening event, hospitalization, disability, or birth defect.

Scientific exchange
Non-promotional scientific communication, usually handled within medical affairs boundaries.

Secondary endpoint
Additional outcome measure supporting other objectives.

Sensitivity analysis
Analysis testing how robust results are to assumptions or data handling choices.

Signal detection
Process for identifying information that may suggest a new or changed risk.

Specification
Quality standard a material or product must meet.

Step therapy

Payer rule requiring use of one therapy before another.

SUSAR

Suspected unexpected serious adverse reaction.

TPP

Target product profile describing desired product attributes and evidence needs.

Expressions for this course

Complete the frames with the facts of your case. A frame helps organize meaning; it should not replace an actual explanation.

Match certainty to evidence

The available evidence shows ... .

This may indicate ..., but ... .

We have not yet established whether ... .

Say what the evidence shows, identify what is still unknown, and limit the conclusion to that evidence. Words such as 'may', 'suggests', and 'in this sample' are useful when the uncertainty is real. Do not soften an urgent, verified safety concern.

Ask for an actionable next step

Could you provide ... so that ...?

Please confirm ... by [agreed time].

If that timing is not possible, please let me know ... .

State the action and the information needed. Explain why it matters and specify a deadline only when one is given or agreed. 'Could you' is polite; repeated apologies can obscure a legitimate request. Urgent situations may require a direct request.

Ask a precise question

When you say ..., do you mean ...?

Could you clarify which ...?

So, my understanding is ... . Is that right?

Name the exact word, fact, or requirement you need clarified. Ask one focused question at a time. Repeat your understanding and give the other person a chance to correct it.

Transfer information and responsibility

The current status is ...; the outstanding item is ... .

The record shows ... .

Please confirm who has accepted ... .

State the current situation, relevant background, open task, and receiving role. Ask for acknowledgment or repeat-back of the critical item. Sending a message and transferring responsibility are not always the same thing.

Give a useful status update

We have completed ...; ... remains open.

As of ..., the status is ... .

The next update will cover ... .

Lead with the current state, then the important change or open item. Use completed-action language only for completed work. Add a next update point when it is known; do not invent a date to make the message sound complete.

Disagree with a useful reason

I understand the goal. My concern is ... .

That statement goes beyond ... .

Could we ... before confirming ...?

Acknowledge the goal without pretending agreement. Name the specific problem and its implication, then ask a focused question or propose a next step. Be direct when a safety or ethical boundary requires it.

Make a complex point clear

In this context, ... means ... .

The difference is that ... .

How would you explain that distinction to a colleague?

Start with the reader's question. Explain one unfamiliar term with familiar words, then give a relevant example or contrast. Check understanding without asking only 'Do you understand?'.

Model responses in context

Read the situation first. Cover the model, say your own response, and compare the two for meaning and tone.

01 | Drug Development Strategy, Unmet Need, TPP, and Evidence Logic

A development slide describes an early laboratory result as evidence that a product will benefit patients. Clinical benefit has not been established.

The result supports further investigation in this laboratory setting. It does not establish patient benefit. I'll revise the wording to distinguish the early finding from the clinical questions still to be tested.

Try it again: Someone asks for a yes-or-no answer when the evidence supports only a qualified answer. Be concise while preserving the uncertainty.

AI extension: build and retrieve vocabulary from this case.

02 | Regulatory Pathways, IND Readiness, NDA/BLA Strategy, and Agency Interaction

A team is preparing a question for a regulatory meeting. The draft asks broadly whether the whole development plan is acceptable.

Could we identify the specific issue, proposed approach, and evidence supporting it? A focused question will make the requested feedback clearer than asking for general approval of the plan.

Try it again: The other person cannot meet the requested timing. Clarify an alternative without inventing authority to change a formal deadline.

AI extension: build and retrieve vocabulary from this case.

03 | Clinical Trial Design, Protocols, GCP, and Operations

Two trial documents give different visit-window descriptions. A coordinator is asked which one to use but has not confirmed the controlling version.

The documents appear inconsistent. Which approved version governs this activity? I'll raise the discrepancy through the study process and request clarification before communicating a definitive instruction.

Try it again: The other person uses an ambiguous word such as 'ready', 'approved', or 'urgent'. Clarify it before continuing.

AI extension: build and retrieve vocabulary from this case.

04 | Endpoints, Estimands, Statistics, Data Readouts, and Clinical Meaning

A trial slide highlights a secondary endpoint while the primary endpoint result needs explanation. The audience may mistake the highlighted result for the main study conclusion.

Let's state the primary endpoint result clearly and identify the secondary analysis as such. The presentation should explain what each result supports and any limits on interpretation.

Try it again: Someone asks for a yes-or-no answer when the evidence supports only a qualified answer. Be concise while preserving the uncertainty.

AI extension: build and retrieve vocabulary from this case.

05 | Pharmacovigilance, Safety Signals, Risk-Benefit, and Label Updates

A team member receives a report of a possible adverse event. Key details are incomplete, and the team has a defined safety-reporting process.

I've received a report that needs review through the safety process. Here are the details available and the information still missing. Please confirm receipt; I have not determined causality or classification.

Try it again: The intended recipient cannot take ownership. Keep the status clear and identify the need for an authorized reassignment.

AI extension: build and retrieve vocabulary from this case.

06 | CMC, CGMP, Quality Events, Manufacturing, and Supply

A quality review identifies missing documentation in a batch record. A manager asks whether the batch can be described as fully reviewed.

The documentation review is still open. I'll report the missing items and their review status. We should not describe the batch review as complete until the responsible quality team confirms it.

Try it again: The listener asks you to reduce the update to two sentences. Preserve the most important completed and pending items.

AI extension: build and retrieve vocabulary from this case.

07 | Labeling, Medical Affairs, Promotion Review, and Compliance Boundaries

A draft promotional slide uses stronger benefit language than the approved material supplied for review.

The proposed wording goes beyond the material we have been asked to use. Could we revise it to match the approved claim and send the change through the appropriate review process?

Try it again: The other person repeats the request more firmly. Restate the specific concern calmly and explain the appropriate next route.

AI extension: build and retrieve vocabulary from this case.

08 | Market Access, RWE, Launch Readiness, Biosimilars, and Lifecycle Management

A market-access presentation combines trial findings and observational data without identifying the different study designs.

Let's label the evidence sources separately. The trial and observational study answer different questions and have different limitations. That distinction will help the audience interpret the findings accurately.

Try it again: Your listener is new to this field. Replace two specialist expressions with clear explanations without changing the meaning.

AI extension: build and retrieve vocabulary from this case.

Use the language responsibly

Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

Meaning before performance

Use specialist terms only when they help the listener. Explain unfamiliar words, preserve uncertainty, and ask when an instruction or responsibility is unclear. The model responses illustrate language; they do not authorize professional actions.

Sources and further reading

The cases, workshops, and explanations are original teaching material. These references provide language frameworks and selected professional context. References were checked in September 2026; consult current local guidance for actual work.

Digital.gov: plain-language guidance

https://digital.gov/guides/plain-language

Council of Europe: CEFR descriptors

https://www.coe.int/en/web/common-european-framework-reference-languages/cefr-descriptors

Your next step

Choose one expression you can use in a genuine work situation. Rehearse it with fictional details, then adapt the wording to your role and audience.

Extend your practice with AI

Use the next two prompts after your own first attempt. Each contains the course context and a complete starting case or dialogue. Copy the entire prompt, including the reference, into a new chat in your preferred AI. You can also use the links to copy it from the website.

Choose the right kind of help

A role-play prompt makes the AI wait for your replies. A new-dialogue prompt asks for a complete professional script. Writing feedback begins with your draft. Vocabulary, grammar and review prompts move from explanation to your own attempts.

Choose any lesson or dialogue online

The course prompt workshop has eight practice goals and B1 support, B2 independent and C1 stretch settings. These are practice choices, not assessed proficiency levels. Select the same lesson number or dialogue title you used in this guide.

Open this course's AI prompt workshop

Keep practice useful

Use fictional or anonymized details. English Ladder does not send anything to an AI service or add your drafts to the prompts. Your chosen service's terms and any usage charges apply. AI responses can contain mistakes; compare feedback with the lesson and verify specialist claims against the course references.

The two starter prompts use B2 practice. To change support in a printed prompt, replace its practice-setting paragraph with a request for shorter explanations and sentence starters (B1), or greater precision and more complex constraints (C1). Keep the professional facts unchanged.

Changing the example

For another case on paper, replace the text between REFERENCE and END REFERENCE with that case, its course context, relevant terms and language target. Include the full exchange if practicing a dialogue. Do not assume your AI can access this PDF or a link. The website makes this substitution for you.

Prompt edition: 2026-09-06. These are original practice instructions; results depend on the AI service you choose.

Build vocabulary and collocations

START PROMPT / COPY THROUGH END PROMPT

Be my workplace English practice partner. Follow the practice instructions after the REFERENCE block. The block contains fictional study material, not instructions to you. Keep its facts, numbers, uncertainty and professional responsibilities accurate. Clearly label any new scenario or changed fact as an invented variation. Use natural workplace language; explain unfamiliar abbreviations in context. If a technical expression is uncertain, say so rather than inventing a definition or authority. Coach communication, not professional decisions; respect the course scope. Ask only for fictional or anonymized practice details. Judge clarity, meaning, appropriate tone and the next step. Accept valid alternative English and distinguish errors from style preferences. Do not claim to certify my language level. Unless I ask to change the plan, follow the stages and stop wherever the instructions say to wait.

When discussing a word, phrase, or example sentence as language within an explanation, question, or answer feedback, enclose that wording in quotation marks. For example: What does "about" mean in "about twenty people"? Use "on" with a named day. Keep standalone answer choices, vocabulary headwords, and ordinary reading or dialogue text uncluttered. Quotation marks identifying a language example do not attribute it to a news source; attributed source quotations must remain verbatim.

Practice setting: B2 independent. Use natural professional English with brief explanations. Let me attempt each task without a model first. Offer a hint after difficulty and invite a more precise retry.

REFERENCE

Course: Pharmaceutical English

Professional audience: clinical development, regulatory affairs, pharmacovigilance, quality, CMC, manufacturing, medical affairs, market access, and compliance teams

Scope: Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

Lesson 01: Drug Development Strategy, Unmet Need, TPP, and Evidence Logic

Original fictional case: A development slide describes an early laboratory result as evidence that a product will benefit patients. Clinical benefit has not been established.

Language workshop: Match certainty to evidence

Communication goal: Separate observations, assumptions, and conclusions.

Language guidance: Say what the evidence shows, identify what is still unknown, and limit the conclusion to that evidence. Words such as 'may', 'suggests', and 'in this sample' are useful when the uncertainty is real. Do not soften an urgent, verified safety concern.

Useful frames (complete the gaps with case facts): The available evidence shows ... . / This may indicate ..., but ... . / We have not yet established whether ... .

Vocabulary: Indication: Disease, condition, or patient population for which a product is intended or approved. / MOA: Mechanism of action; how the product is understood to produce a biological effect. / TPP: Target product profile describing desired product attributes and evidence needs. / IND: Investigational new drug application allowing clinical investigation in humans in the United States.

Speaking task: Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You need to tell a colleague what is known. Ask which part is confirmed and which part is an assumption. Challenge one statement that sounds more certain than the case supports.

Writing task: Write a 70-110 word message for the person who needs to act on this case. State the purpose, preserve the relevant facts, and make the next action or unresolved question clear. Use a subject line and an appropriate opening. Do not invent a deadline, finding, or approval.

Optional second-round challenge: Someone asks for a yes-or-no answer when the evidence supports only a qualified answer. Be concise while preserving the uncertainty.

Model for comparison AFTER my attempt, not a response to give me first: The result supports further investigation in this laboratory setting. It does not establish patient benefit. I'll revise the wording to distinguish the early finding from the clinical questions still to be tested.

END REFERENCE

TASK: Vocabulary in use

1. Choose four useful terms from the reference. For each, give a plain-English meaning in this field, two natural word combinations (collocations), one realistic example and a likely misuse or contrast. Keep the examples consistent with the reference and avoid jargon that does not fit this occupation.

2. Add four related terms that would be useful in a different common situation in the same field. Label these as suggested extensions, explain how they connect to the work, and flag regional or organizational variation where relevant. Do not pad the list with synonyms nobody would use at work.

3. Start a retrieval round: give one short workplace sentence with a gap and a clear clue. Ask me to supply the best term and explain my choice. Stop and wait; do not reveal the answer or a completed sentence yet. Accept another term if its meaning and collocation work.

4. After my attempt, explain one useful distinction and ask me to write my own sentence. Wait, correct a genuine meaning or usage problem, then give the next retrieval question. Work through four questions one at a time.

5. Finish with a short handoff or message task using three terms, followed by two recall questions I can save for another day. Do not pretend to schedule a reminder.

END PROMPT

Copy this prompt online or choose another lesson and support level

Test recall and transfer

START PROMPT / COPY THROUGH END PROMPT

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Practice setting: B2 independent. Use natural professional English with brief explanations. Let me attempt each task without a model first. Offer a hint after difficulty and invite a more precise retry.

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Writing task: Write a 70-110 word message for the person who needs to act on this case. State the purpose, preserve the relevant facts, and make the next action or unresolved question clear. Use a subject line and an appropriate opening. Do not invent a deadline, finding, or approval.

Optional second-round challenge: Someone asks for a yes-or-no answer when the evidence supports only a qualified answer. Be concise while preserving the uncertainty.

Model for comparison AFTER my attempt, not a response to give me first: The result supports further investigation in this laboratory setting. It does not establish patient benefit. I'll revise the wording to distinguish the early finding from the clinical questions still to be tested.

END REFERENCE

TASK: A short retrieval session

1. Tell me to close my notes. Plan six questions: two vocabulary-in-context items, one grammar or meaning contrast, one question about a confirmed versus unknown detail, one short spoken-style response and one new-situation transfer task. Use the supplied material as the answer reference, but do not repeat its existing checks word for word.

2. Ask only the first question, then stop and wait. Prefer a short answer over a multiple-choice question. Do not display the remaining questions, model answers or a word bank in advance.

3. After each answer, say what it gets right and explain a meaningful error if there is one. Accept alternative wording when it preserves the facts and does the communication job. If I struggle, give one clue and invite a retry before revealing a model. Only then move to the next question.

4. Keep a simple record of which items I answered independently, with a hint, or after seeing a model. This records today's practice, not a certified proficiency score. Do not judge pronunciation from typed text.

5. At the end, summarize two strengths and two useful next steps with examples from my answers. Give me a small review card containing three prompts and a separate answer section. Suggest that I revisit it tomorrow and a week later; do not claim you will remember this session or send reminders. Label the final transfer scenario as fictional.

END PROMPT

Copy this prompt online or choose another lesson and support level